SACTCHECK · NCCP 00232

# My complete treatment handbook

## Vinorelbine IV · non-small-cell lung cancer

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

## Contents


1. Your three-page treatment companion — page 2
2. Full treatment reference: medicines, schedule, tests, symptoms and follow-up — page 5
3. Detailed treatment and monitoring plan — page 19
4. Supportive medicines and care — page 22
5. Visits, access and practical arrangements — page 23
6. Your treatment passport and symptom record — page 24
7. Wallet card and phone instructions — page 29


SACTCheck v0.84.31 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

# Vinorelbine IV · non-small-cell lung cancer

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

## My overall treatment

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/ unacceptable toxicity; team review determines continuation.

**Treatment**
Days 1 and 8; repeat every 21 days. Duration agreed with your team.

**Review**
Your team reviews benefit and side effects, then agrees whether to continue, pause or change treatment.

## A typical 21-day cycle

**Day 1–21**
One reference cycle

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

## My tests and checks

| When | What is checked | Timing / reason |
|---|---|---|
| **Before** | **Blood count, kidneys / liver, nerves**<br>Required by regimen | Before first treatment |
| **During** | **Blood count**<br>Required by regimen | On the day of every administration, including Day 8 (product information) |
| **During** | **Kidneys / liver and nerves**<br>Required by regimen | Before each 21-day cycle |
| **After / review** | **Scans, markers and recovery**<br>Team-agreed plan | Individual plan; no fixed scan or tumour-marker interval |

## Between treatments

- Vinorelbine is planned on Days 1 and 8; there is no routine dose in the third week.
- Constipation and numbness may build up. Follow your bowel plan and report changes before dosing.

Bring your medicine list and report new symptoms before dosing. Confirm the prescription, test appointments and daytime / out-of-hours contact plan.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE NUMBERS ONLY — your team must replace these before patient use.**

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Blood counts and infection

**Vinorelbine**

Low blood counts can cause infection, tiredness, bruising or bleeding. Fever, chills or sudden illness: call oncology immediately.

### 3 Tingling, numbness or weakness

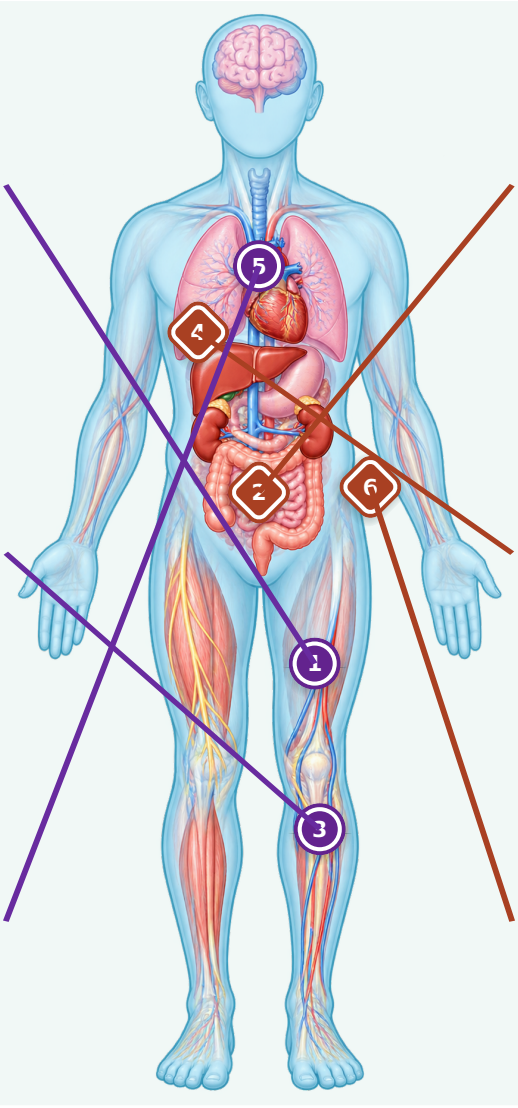
**Vinorelbine**

Nerve effects may change sensation, grip, walking or bowel movement. Report changes early because the next dose may need to wait or be reduced.

### 5 Uncommon serious symptoms

**Vinorelbine**

New chest pain, sudden or worsening breathlessness, or severe allergic symptoms need urgent medical help. Previous heart disease and interacting medicines need review before treatment.




### 2 Bowel and mouth symptoms

**Vinorelbine**

Constipation, vomiting or sore mouth can occur. Severe pain, swelling, repeated vomiting or no stool/gas: urgent assessment; do not take extra laxatives.

### 4 Fatigue, hair and liver tests

**Vinorelbine**

Tiredness, hair thinning/loss and liver-test changes can occur. Pace activities and tell the team if eating, drinking or daily tasks become difficult.

### 6 Infusion site

**Vinorelbine**

Tell staff immediately about burning, pain, swelling or leakage.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/ obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE NUMBERS ONLY — your team must replace these before patient use.**

# Keep your treatment details to hand

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Vinorelbine IV**

Days 1 and 8; repeat every 21 days. Duration agreed with your team.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE NUMBERS ONLY — your team must replace these before patient use.**

Hospital / team: _____ Backup contact route: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.31 · NCCP 00232 v5 · 3 / 3
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

NCCP 00232 · VERSION 5

# Your treatment, explained

Vinorelbine IV · non-small-cell lung cancer

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

## When to get help

### Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

### Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

**Open your CUH contact / treatment card**

Sources: nccp · smpc

**Choose a topic to read**

YOUR COMPLETE REFERENCE

## Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.


- How each drug works
- The phases and how they connect
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- The treatment being discussed
- Checks before and during treatment
- Support and safety at home
- Supportive medicines
- Questions to ask
- Sources


UNDERSTAND YOUR TREATMENT

# How the medicines work

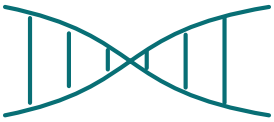
Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side

effects, previous treatments and your priorities, and reviews whether to continue.

VINCA ALKALOID CHEMOTHERAPY

## Vinorelbine

IV infusion



**Disrupts the machinery of cell division**

Vinorelbine disrupts the tiny structures a cell needs to divide. This can stop cancer cells multiplying. Healthy blood-forming cells, gut cells and nerves can also be affected.

Binds cell-division structures → Microtubules cannot function normally → Cell division is disrupted

### What to watch for

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

**Explore your existing anatomy and side-effect map**

THE BIGGER TREATMENT PLAN

# The phases and how they connect

**21-DAY REFERENCE**

### Vinorelbine IV · non-small-cell lung cancer

**21-day interval**

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

These are the medicines in this exact schedule. Dose holds, missed tablets and omitted visits need instructions from your treating team.

Sources: nccp

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**BASELINE → TREATMENT → FOLLOW-UP**

## When checks happen

Milestones are not equal time intervals. Your team books and reviews tests. Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

| Check | Before starting | Early treatment | Ongoing treatment | Dose change / pause | Next decision |
|---|---|---|---|---|---|
| **Blood / kidney / liver** | Baseline blood and organ tests | Day 1 and Day 8 blood counts | Counts before every dose; organ tests each cycle | Review recovery and current dose | Follow unresolved abnormalities |
| **Symptoms / medicines** | Bowel / nerve / infection review | Report early symptoms and support problems | Review all medicines and side effects | Do not restart or escalate yourself | Agree continuation and response assessment |
| **Special schedule check** | Confirm Day 1 and Day 8 visits | Check Day 8 clearance | Continue 21-day pattern | Keep previous dose / nadir history | Personal plan; no booked dates implied |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00232 v5 · official regimen · HPRA Navelbine IV SmPC, 8 January 2024

**THE WHOLE TREATMENT JOURNEY**

# Vinorelbine IV · non-small-cell lung cancer: treatment and checks

### Non-small-cell lung cancer · IV monotherapy

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Early treatment | Ongoing treatment | Dose change / pause | Next decision |
|---|---|---|---|---|---|
| **Treatment plan** NCCP 00232 | Confirm lung monotherapy plan and current dose | Days 1 and 8 each 21-day cycle | Continue while helpful and tolerated | Review dose and recovery before restart | Agree continuation / next treatment |
| **Blood / kidney / liver** NCCP 00232 · SMPC | Baseline blood and organ tests | Day 1 and Day 8 blood counts | Counts before every dose; organ tests each cycle | Review recovery and current dose | Follow unresolved abnormalities |
| **Symptoms / medicines** NCCP 00232 · SMPC | Bowel / nerve / infection review | Report early symptoms and support problems | Review all medicines and side effects | Do not restart or escalate yourself | Agree continuation and response assessment |
| **Special schedule check** NCCP 00232 · SMPC | Confirm Day 1 and Day 8 visits | Check Day 8 clearance | Continue 21-day pattern | Keep previous dose / nadir history | Personal plan; no booked dates implied |

## What each check is for

**1**

### Treatment and dose review

Before every administration

Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

**Ask / act:** The treating team confirms results, dose and suitability; this map does not authorise treatment.

Sources: NCCP 00232 · SMPC

**2**

### Support and symptoms

Throughout treatment, including days at home

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

**Ask / act:** Keep a written medicine plan and urgent-contact route.

Sources: NCCP 00232 · ORAL-ANTIEMETIC · NCIS

**3**

### Response and continuation

At agreed clinical review points

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

**Ask / act:** Confirm the current plan with the team.

Sources: NCCP 00232 · TRIAL

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?

- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

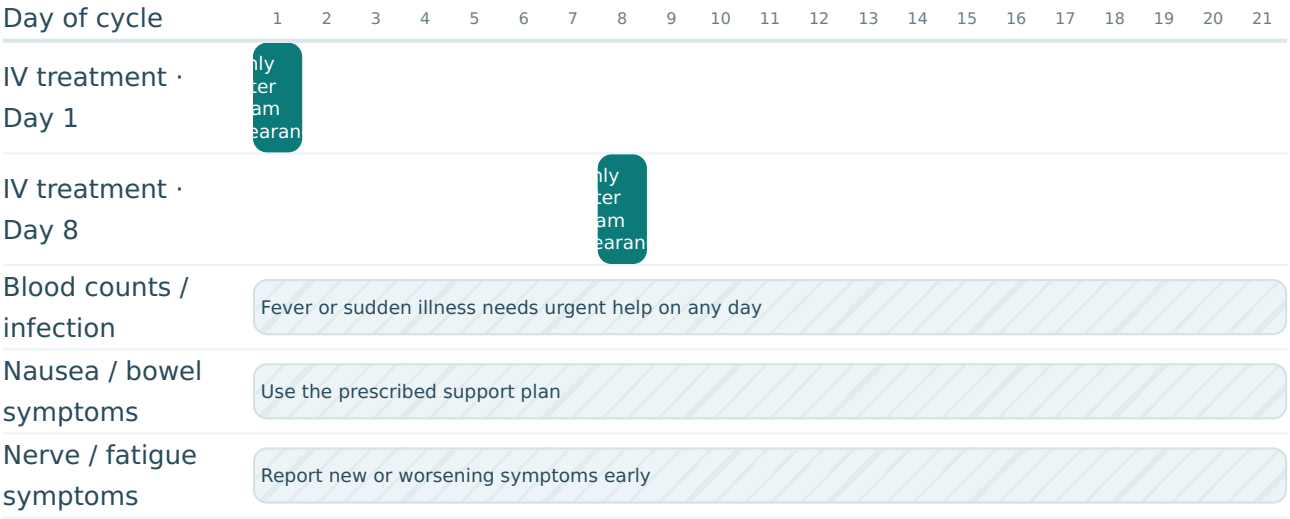
Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

# Vinorelbine IV · non-small-cell lung cancer

## Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.




**Treatment and recovery:** 30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

**Tests and decisions:** Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

> **Why these bands stay broad:** This is a relative reference, not an appointment calendar. Treatment days can change after clinical review. Symptom bands do not predict when an effect will occur. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00232 v5 · official regimen ↗

**There are no guaranteed “safe days”.** A day without treatment is not automatically a safe day. Use the emergency contact when needed; do not move or restart doses yourself.

**Read the whole cycle as text**

### Day 1–21: One reference cycle

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation. Follow the written plan from your treatment team; no dates are entered here.

PRACTICAL TREATMENT GUIDE

## Your practical treatment plan

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.



STEP 1

### Visits and tests

Two infusion visits per 21-day cycle, with blood tests before doses. Peripheral cannula or existing access according to the treating service; a port is not automatically required.



STEP 2

### Treatment preparation

Wear clothing allowing IV-site observation; report discomfort immediately.



STEP 3

### At home

No home vinorelbine capsules are part of this regimen. Keep an up-to-date medication list and emergency contact.

**Individual instructions**

NCCP specifies 50 mL sodium chloride 0.9% over 15 minutes followed by 250 mL saline. Current Navelbine SmPC says 6–10 minutes: product/local-pharmacy reconciliation is required. Do not promise a total visit length; preparation, tests and observation add time.

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**Open practical treatment guide** · **Printable practical guide**

Sources: NCCP 00232 v5 · official regimen · HPRA Navelbine IV SmPC, 8 January 2024. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

# Support around the medicines

### 1 Sickness plan

No routine preventive anti-sickness medicine is usually required for this IV regimen. Ask for a rescue plan and report uncontrolled symptoms.

### 2 Bowel care

Agree a constipation plan, particularly if using opioids or ondansetron. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment; do not simply add laxatives.

### 3 Medicines and vaccines

Check every new prescription, supplement and vaccine with the team; avoid grapefruit. Live vaccines and interacting medicines need review.

Sources: nccp · oral-antiemetic · ncis

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TREATMENT REFERENCE · 01

# The treatment being discussed

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side

effects, previous treatments and your priorities, and reviews whether to continue.

**Lung treatment context**

NCCP 00232b: non-small-cell lung cancer. This guide covers vinorelbine monotherapy. Platinum combinations, adjuvant treatment and radiotherapy combinations need their own complete plans; this guide does not authorise them.

Sources: nccp

**Schedule and duration**

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

Sources: nccp

**How treatment is given**

NCCP specifies 50 mL sodium chloride 0.9% over 15 minutes followed by 250 mL saline. Current Navelbine SmPC says 6–10 minutes: product/local-pharmacy reconciliation is required. Do not promise a total visit length; preparation, tests and observation add time.

Sources: nccp

TREATMENT REFERENCE · 02

# Checks before and during treatment

Every dose depends on the reviewed treatment plan and results.

### Blood and symptom checks

Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

Sources: nccp

### Heart and response checks

Review cardiovascular history and new symptoms. These monotherapy NCCP protocols do not mandate the serial LVEF/echo schedule used for anthracyclines or trastuzumab; arrange cardiac tests when clinically indicated.

Sources: nccp

### Fertility, pregnancy and breastfeeding

Discuss the disease-control aim, likely benefit and uncertainty, other lung-cancer treatments or symptom-focused care, and the burden of visits and tests. Review low blood counts and infection, nausea/vomiting, fatigue, constipation/ileus, nerve symptoms, hair changes and uncommon serious lung/heart/allergic effects. Discuss IV-site injury and the different IV and capsule doses; the product-specific infusion plan needs pharmacy confirmation. Discuss fertility preservation and pregnancy planning. Do not breastfeed. Confirm the exact product contraception advice: HPRA recommends contraception during treatment and for 7 months afterwards for women, and 4 months for men.

Sources: smpc

TREATMENT REFERENCE · 03

# Support and safety at home

Keep the written medicine and urgent-contact plan.

### Sickness plan

No routine preventive anti-sickness medicine is usually required for this IV regimen. Ask for a rescue plan and report uncontrolled symptoms.

Sources: nccp

### Bowel care

Agree a constipation plan, particularly if using opioids or ondansetron. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment; do not simply add laxatives.

Sources: nccp

### Medicines and vaccines

Check every new prescription, supplement and vaccine with the team; avoid grapefruit. Live vaccines and interacting medicines need review.

Sources: nccp

### Urgent contact

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Sources: nccp

## Why this treatment may be chosen

Studies support vinorelbine as an option in selected people with advanced non-small-cell lung cancer. The strength and type of evidence differ for the IV and capsule schedules.

The team considers tumour tests, previous treatment, health, symptoms and your preferences. Results from a group cannot predict your own benefit.

Sources: nccp · trial

PREPARE FOR THE CONVERSATION

# Questions worth asking

**Can IV and oral vinorelbine be exchanged?**

For IV use only; intrathecal administration may be fatal. Oral and IV vinorelbine are not interchangeable mg-for-mg. Stop infusion and tell staff immediately about pain, burning, swelling or leakage at the IV site; extravasation can damage tissue.

**How long will treatment continue?**

Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

**Do I need regular heart scans?**

Review cardiovascular history and new symptoms. These monotherapy NCCP protocols do not mandate the serial LVEF/echo schedule used for anthracyclines or trastuzumab; arrange cardiac tests when clinically indicated.

**How long is an infusion visit?**

The source describes a short infusion followed by a saline flush. Your unit confirms the product-specific infusion instructions and complete visit length, including blood results, preparation and observation.

> **Try explaining the plan back:** Which treatment days and doses are prescribed, which blood tests must happen first, how do you take the support medicines, and which symptoms need urgent help?

SOURCE TRANSPARENCY

# Where this information comes from

Regimen schedule: NCCP 00232 v5. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00232 v5 · official regimen
- HPRA Navelbine IV SmPC, 8 January 2024
- NCCP antiemetic guidance v6
- NCCP NCIS antiemetic medicines v8
- ELVIS Group, JNCI 1999: IV vinorelbine versus supportive care in older adults with advanced NSCLC

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SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

## Written source addresses

https://healthservice.hse.ie/documents/6567/232_v5_Vinorelbine_IV.pdf

https://assets.hpra.ie/products/Human/26035/Licence_PA0329-011-003_08012024110935.pdf

https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/9890172/

BASELINE → TREATMENT → FOLLOW-UP

# When checks happen

Milestones are not equal time intervals. Your team books and reviews tests. Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

| Check | Before starting | Early treatment | Ongoing treatment | Dose change / pause | Next decision |
|---|---|---|---|---|---|
| **Blood / kidney / liver** | Baseline blood and organ tests | Day 1 and Day 8 blood counts | Counts before every dose; organ tests each cycle | Review recovery and current dose | Follow unresolved abnormalities |
| **Symptoms / medicines** | Bowel / nerve / infection review | Report early symptoms and support problems | Review all medicines and side effects | Do not restart or escalate yourself | Agree continuation and response assessment |
| **Special schedule check** | Confirm Day 1 and Day 8 visits | Check Day 8 clearance | Continue 21-day pattern | Keep previous dose / nadir history | Personal plan; no booked dates implied |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00232 v5 · official regimen · HPRA Navelbine IV SmPC, 8 January 2024

THE WHOLE TREATMENT JOURNEY

# Vinorelbine IV · non-small-cell lung cancer: treatment and checks

### Non-small-cell lung cancer · IV monotherapy

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Early treatment | Ongoing treatment | Dose change / pause | Next decision |
|---|---|---|---|---|---|
| **Treatment plan** NCCP 00232 | Confirm lung monotherapy plan and current dose | Days 1 and 8 each 21-day cycle | Continue while helpful and tolerated | Review dose and recovery before restart | Agree continuation / next treatment |
| **Blood / kidney / liver** NCCP 00232 · SMPC | Baseline blood and organ tests | Day 1 and Day 8 blood counts | Counts before every dose; organ tests each cycle | Review recovery and current dose | Follow unresolved abnormalities |
| **Symptoms / medicines** NCCP 00232 · SMPC | Bowel / nerve / infection review | Report early symptoms and support problems | Review all medicines and side effects | Do not restart or escalate yourself | Agree continuation and response assessment |
| **Special schedule check** NCCP 00232 · SMPC | Confirm Day 1 and Day 8 visits | Check Day 8 clearance | Continue 21-day pattern | Keep previous dose / nadir history | Personal plan; no booked dates implied |

# What each check is for

## 1 Treatment and dose review

Before every administration

Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

**Ask / act:** The treating team confirms results, dose and suitability; this map does not authorise treatment.

Sources: NCCP 00232 · SMPC

## 2 Support and symptoms

Throughout treatment, including days at home

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

**Ask / act:** Keep a written medicine plan and urgent-contact route.

Sources: NCCP 00232 · ORAL-ANTIEMETIC · NCIS

## 3 Response and continuation

At agreed clinical review points

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

**Ask / act:** Confirm the current plan with the team.

Sources: NCCP 00232 · TRIAL

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

# Vinorelbine IV · non-small-cell lung cancer

**Treatment scope · NCCP 00232** Lung monotherapy context only. The breast guide and its evidence remain separate. Discuss the exact disease setting and alternatives with the treating team.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Vinorelbine IV · non-small-cell lung cancer · support plan** — Minimal IV risk
  Vinorelbine

https://sactcheck.com/patient/00232/lung/supportive-care.html

# Vinorelbine IV · non-small-cell lung cancer

NCCP 00232 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

## Your practical treatment plan

30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

STEP 1

### Visits and tests

Two infusion visits per 21-day cycle, with blood tests before doses. Peripheral cannula or existing access according to the treating service; a port is not automatically required.

STEP 2

### Treatment preparation

Wear clothing allowing IV-site observation; report discomfort immediately.

STEP 3

### At home

No home vinorelbine capsules are part of this regimen. Keep an up-to-date medication list and emergency contact.

### Individual instructions

NCCP specifies 50 mL sodium chloride 0.9% over 15 minutes followed by 250 mL saline. Current Navelbine SmPC says 6–10 minutes: product/local-pharmacy reconciliation is required. Do not promise a total visit length; preparation, tests and observation add time.

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Sources: NCCP 00232 v5 · official regimen · HPRA Navelbine IV SmPC, 8 January 2024. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

# Vinorelbine IV · non-small-cell lung cancer

## A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

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The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

**Emergency: 112 / 999.** Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

**Contact oncology immediately** for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital’s instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your out-of-hours pathway / nearest ED**

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00232/lung/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

# Vinorelbine IV · non-small-cell lung cancer

Cycle number: _____ Start date: _____ Next review: _____

**Vinorelbine IV · non-small-cell lung cancer · reusable record** · copy this sheet for another cycle of the same phase.

### Vinorelbine · Vinca alkaloid chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Fatigue** | ☐ Usual energy | ☐ Tiredness improves with rest | ☐ Rest does not help; usual tasks affected: call | ☐ Self-care difficult: urgent review | ☐ Collapse: 112/999 |
| **Nausea** | ☐ None | ☐ Queasy but eating and drinking | ☐ Intake reduced: contact the team | ☐ Unable to maintain intake: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Bleeding / bruising** | ☐ None | ☐ New small bruise / brief bleed: report | ☐ Repeated bleeding: call today | ☐ Bleeding that will not stop: urgent help | ☐ Heavy bleeding / collapse: 112/999 |
| **Numbness / tingling** | ☐ None | ☐ Mild change; usual tasks unaffected | ☐ Limits buttons, writing or walking: call | ☐ Washing, dressing or walking difficult: urgent review | ☐ Sudden major weakness / paralysis: emergency help |
| **Constipation** | ☐ Usual bowel pattern | ☐ Mild change: follow the agreed bowel plan | ☐ Persistent constipation or discomfort: contact the team | ☐ Pain, vomiting or no stool/wind: urgent assessment | ☐ Collapse or severe sudden illness: emergency help |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

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# Keep your treatment details to hand

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Vinorelbine IV**

Days 1 and 8; repeat every 21 days. Duration agreed with your team.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE NUMBERS ONLY — your team must replace these before patient use.**

Hospital / team: _____ Backup contact route: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.31 · NCCP 00232 v5 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.